

The British Sector of the Western Front, 1914–1918

The Historic Environment · Injuries, Treatment and the Trenches · Complete Mastery & Source Studio

Pupil Name: _____

Class / Set: _____

Target Grade: []

SECTION A PROGRESS & ASSESSMENT TRACKER (16 MARKS TOTAL)

Marked by Teacher or Peer Verified

Assessment Component & Stem Focus	Format Style	Max Marks	Pupil Score
Round 1 (Stepped): Q1(a) Feature 1 (Casualty Clearing Stations)	Stepped Ladder	/2	_____
Round 1 (Stepped): Q1(b) Feature 2 (Thomas Splint)	Stepped Ladder	/2	_____
Round 1 (Stepped): Q2(b) 4-Part Follow-Up Grid (Base Hospital Water)	Stepped Ladder	/4	_____
Round 1 (Stepped): Q2(a) Source Utility (Somme CCS vs Base Hospital)	Stepped Ladder	/8	_____
Round 2 (Dual-Track): Q1(a) & Q1(b) Features (Mobile X-rays & Trench Fever)	Dual Track	/4	_____
Round 2 (Dual-Track): Q2(b) 4-Part Follow-Up Grid (Gas Casualties)	Dual Track	/4	_____
Round 2 (Dual-Track): Q2(a) Source Utility (RAMC Logbook & Flanders Mud)	Dual Track	/8	_____
Round 3 (Simulation): Full Section A Timed Exam Pitch (Q1a, Q1b, Q2a, Q2b)	Exam Hall (25m)	/16	_____
TOTAL COMBINED EXAM PRACTICE MARKS:		/48	_____ / 48

Teacher / Peer Marker: _____

Date: _____

Overall Grade: [9 8 7 6 5 4]

SECTION A EDEXCEL EXAM FAST FACTS & STEM BLUEPRINTS (16 MARKS · ~25 MINS) OPTION 11 PAPER 1 SECTION A

- **Q1(a) & Q1(b) Features (2 Marks each · ~3 mins each):** *F-D Formula*. State **Feature** in sentence 1 → support with **1 precise supporting factual detail**. (2 separate questions).
- **Q2(a) Source Utility (8 Marks · ~12 mins):** *C-O-P Matrix*. Evaluate both sources for **Content** (what it says) → **Own Contextual Knowledge** → **Provenance** (Nature, Origin, Purpose / NOP).
- **Q2(b) Follow-Up Investigation (4 Marks · ~7 mins):** *4-Step Enquiry*. Quote **Detail in Source B** → Formulate **Targeted Question** → Name **Specific Historical Source Type** → Explain **Direct Purpose**.

SECTION A: THE HISTORIC ENVIRONMENT

Question 1 (a): Feature Question (2 Marks)

F-D Formula · ~3 Mins

Describe one feature of the work of Casualty Clearing Stations (CCS) on the Western Front.

 **Level 1 Launchpad:** Starter: "One feature of Casualty Clearing Stations was that..."
Facts to Include: Located 7–12 miles back near railways; triage (walking wounded, urgent surgery, dying); performed critical amputations and chest surgeries before base evacuation.

One feature of Casualty Clearing Stations was that...

Specifically, (add one precise historical detail or statistic)...

Question 1 (b): Feature Question (2 Marks)

F-D Formula · ~3 Mins

Describe one feature of the use of the Thomas splint on the Western Front.

 **Level 1 Launchpad:** Starter: "One feature of the Thomas splint was that..."
Facts to Include: Introduced in 1915 by Robert Jones; rigid frame pulled leg straight to stop bone ends grinding; dramatically dropped compound fracture mortality from 80% to 20%.

One feature of the Thomas splint was that...

Specifically, (add one precise historical detail or statistic)...

Question 2 (b): Follow-Up Investigation Grid (4 Marks)

4-Step Enquiry · ~6 Mins

Study Source B (Edith Smith letter: "The pipes freeze entirely, meaning we must strictly ration the water provided to the wounded."). How could you follow up Source B to find out more about the severe conditions faced by medical staff? Complete the table below.

Detail in Source B that I would follow up:	"The pipes freeze entirely, meaning we must strictly ration the water provided to the wounded."
Question I would ask:	What specific impact did water shortages have on surgical sterilization and infection rates in winter 1917?
What type of source I would use:	Base Hospital Daily Medical Superintendent Logbooks and Sanitary Inspection Reports (1917–18).
How this source would help me answer my question:	It would provide official daily records of sanitation breakdowns and hospital infection rates during freezing months.

Question 2 (a): Source Utility (8 Marks)

C-O-P Formula · ~12 Mins

Study Sources A and B. How useful are Sources A and B for an historian studying the immense challenges of treating casualties on the Western Front? Explain your answer, using Sources A and B and your knowledge of the historical context.

Source A (Chaplain Arthur Davies, Somme CCS, July 1916):

"The casualties have started to arrive... The volume of patients is overwhelming; every bed is occupied, and men are fortunate just to find a spot on the bare ground. Many will pass away before surgery is possible. We have admitted 1,500 casualties, and the stream has not stopped."

Source B (Edith Smith, VAD Nurse, Base Hospital, Dec 1917):

"Our morning duties begin at 3.30 am! The freezing temperatures are unbearable. Icicles hang thick over ward windows. Even kettles, rubber hot water bottles, and sponges have frozen solid. The pipes freeze entirely, meaning we must strictly ration water provided to the wounded."

PROVENANCE CLUES (Author, Audience, Motive): Who wrote each source (eyewitness chaplain at CCS vs frontline VAD nurse at base hospital)? What was their motive (private diary recording sheer volume vs private letter home detailing physical conditions)? How does their perspective make the sources useful despite subjective emotional focus?

PARAGRAPH 1: UTILITY OF SOURCE A (CONTENT + PROVENANCE + CONTEXT)

Source A is useful for investigating casualty challenges because the content reveals...

From my own knowledge, this accurately reflects the Somme offensive because...

Furthermore, the provenance makes it useful because as a private chaplain's diary...

PARAGRAPH 2: UTILITY OF SOURCE B & COMPARATIVE JUDGEMENT (CONTENT + PROVENANCE + CONTEXT)

Source B is also useful because it highlights the severe environmental difficulties...

Specifically, my own knowledge confirms that Base Hospitals in winter 1917...

The provenance enhances its utility because as a private letter from a VAD nurse...

Overall, both sources are mutually useful because together they demonstrate...

Level 3 (Grade 9) Utility Glue: Weigh up typicality vs limitation — note that Source A shows peak crisis conditions on 1 July 1916 (57,000 casualties in one day), while Source B reflects seasonal winter extremes at the coast.

Questions 1(a) & 1(b): Features (2 + 2 = 4 Marks)

Independent Toolkit

 Fact Vault (AO1)

- **Mobile X-rays:** 6 mobile units in British sector; located at CCS; couldn't detect clothing fragments; tubes overheated.
- **Trench Fever:** Caused by body lice; flu-like pain in legs; affected ~15% of men; led to delousing stations and bathhouses.

 **Trap:** Do not confuse Trench Fever (lice) with Trench Foot (underground mudholes).

Question 2(b): Follow-Up Practice (4 Marks)

Study this extract from an RAMC officer at Ypres: "The chlorine gas casualties stumbled in clutching their throats. We washed their blinded eyes with bicarbonate of soda solutions, but many died from asphyxiation."

[Historical Records Directory](#)

Detail in Source to follow up:	"We washed their blinded eyes with bicarbonate of soda solutions..."
Question I would ask:	How effective were early chemical antidotes compared to later respirator gas masks?
What type of source I would use:	RAMC Field Ambulance Casualty Admission Books & War Office Gas Defence Bulletins (1915).
How this source would help me:	It would record official medical survival rates of gas victims before and after the issue of Small Box Respirators.

Question 2(a): Source Utility Independent Execution (8 Marks) Wartime Trauma & Gas Warfare

How useful are Source C (RAMC Logbook, 1915) and Source D (Official Training Manual on Trench Foot, 1916) for an enquiry into the prevention of trench illness on the Western Front?

 Provenance Evaluation Matrix

Official Reports: High factual accuracy regarding procedures; may minimize failure to maintain morale.

Doctor/Nurse Accounts: Direct clinical insight into actual suffering; localized to one specific sector.

 Causal & Utility Connectives

- "The content is highly useful because it reveals..."
- "The provenance strengthens its value because..."
- "However, the source is limited in scope because..."

 **Level 3 Standard:** Explicitly evaluate how the author's professional role and purpose shape what was included or omitted.

PARAGRAPH 1: UTILITY OF SOURCE C

Source C is useful for an enquiry into prevention because...

Its provenance as an RAMC logbook means...

PARAGRAPH 2: UTILITY OF SOURCE D & SYNTHESIS

Source D is useful in a different way because it shows official policy on...

However, an official manual may not reflect actual frontline reality because...

Comparing both sources, an historian learns that...

SECTION A: THE HISTORIC ENVIRONMENT

Round 3: Timed Exam Simulation · Stems 1(a), 1(b) Features & 2(b) Follow-Up (8 Marks · 12 Mins)

TIMED PITCH PART 1

⚡ PRE-FLIGHT PLANNING ENGINE · FEATURES & FOLLOW-UP (8 MARKS · ~12 MINS)

1. Q1 Features (4m · ~6 mins):
F-D Formula: 1 feature + 1 specific fact. (e.g. Arras: 700 beds/electricity; Cambrai: Robertson blood bank).

2. Q2(b) Follow-Up (4m · ~6 mins):
Quote exact detail → Targeted enquiry question → Primary archival record (RAMC War Diaries) → Direct purpose.

OPTION 11 BLUEPRINT

Question 1(a) (2 Marks): Describe one feature of the underground hospital at Arras:

One feature of the underground hospital at Arras was...

Question 1(b) (2 Marks): Describe one feature of blood transfusion techniques on the Western Front:

One feature of blood transfusion techniques was...

Question 2(b) (4 Marks): Study Source B (Edith Smith letter on frozen pipes). How could you follow up Source B to find out more about the severe conditions faced by medical staff?

Detail in Source B to follow up:	<u>"The pipes freeze entirely, meaning we must strictly ration the water provided to the wounded."</u>
Question I would ask:	
What type of source I would use:	
How this source would help me:	

Question 2(a): Source Utility (8 Marks)

C-O-P Matrix · ~13 Mins

Study Sources A and B. How useful are Sources A and B for an enquiry into the immense difficulties of casualty evacuation on the Western Front? Explain your answer, using Sources A and B and your knowledge of the historical context.

Source A (RAMC Stretcher Bearer, Ypres Salient, Oct 1917):
"The mud was waist-deep in places. Duckboards were blown to splinters by artillery fire. It took four of us six agonizing hours to carry a single stretcher back from the front line to the Advanced Dressing Station, slipping constantly into shell craters."

Source B (Official War Office Medical Evacuation Report, 1917):
"The evacuation chain relies upon motor ambulance convoys from Dressing Stations to railheads. However, in heavy bombardments, roads become impassable. Canal barges along the Yser and railway ambulance carriages equipped with heating stoves must be prioritized."

PROVENANCE CLUES (Author, Audience, Motive): Contrast a frontline stretcher bearer's immediate visceral journal (recording physical exhaustion) with an official military report (recording logistical systems and infrastructure).

PARAGRAPH 1: UTILITY OF SOURCE A (CONTENT + CONTEXT + PROVENANCE)

Source A is useful for investigating casualty evacuation because the content reveals...

From my own knowledge of the Western Front, this reflects the conditions of...

Furthermore, the provenance makes it useful because as a frontline stretcher bearer...

PARAGRAPH 2: UTILITY OF SOURCE B & COMPARATIVE JUDGEMENT

Source B is also useful because it highlights the wider logistical challenges of...

Specifically, my own knowledge confirms that motor ambulances and canal barges...

The provenance enhances its utility because as an official War Office report...

Overall, comparing both sources, an historian learns that while Source A reveals...

...Source B provides the broader strategic perspective on the evacuation chain.

TOP 3 FATAL EXAMINER TRAPS TO AVOID FOR SECTION A

• The "Generic Source" Trap

Never write "I would check a diary" or "the internet" in Q2(b). You must specify an authentic record type, like RAMC War Diaries or Hospital Admission Registers.

• The "Treachery of Bias" Trap

Never reject a source simply because "it is biased so it's useless". Sources written with strong emotion or wartime censorship are highly useful for showing attitudes!

• The Single Feature Trap

Remember Q1 is split into two questions: 1(a) and 1(b). Do not combine them into one paragraph. Each requires one identified feature + one specific factual detail.

SPECIFICATION PRACTICE BANK: 100% CURRICULUM COVERAGE

Every remaining Western Front specification bullet point tested below

- 1. Features (2m):** Describe one feature of the role of the FANY (First Aid Nursing Yeomanry) in ambulance transport.
- 2. Features (2m):** Describe one feature of the effects of mustard gas on soldiers.
- 3. Source Utility (8m):** How useful are official trench casualty statistics compared to private letters for studying the effectiveness of Brodie helmets?
- 4. Features (2m):** Describe one feature of the Carrel-Dakin method for treating gas gangrene.
- 5. Follow-Up (4m):** Detail: "*Ambulance barges along the Somme-Yser canal were fitted with spring beds.*" Formulate a question and name a primary source.

⚡ **QUICK-FIRE FOLLOW-UP SOURCE DRILL:** When asked to research: (1) *Wound infections* → RAMC Medical Officer Daily Logs; (2) *Frontline delousing routines* → Battalion War Diaries; (3) *Blood transfusions* → CCS Surgical Register.

Medicine in Britain, c1250–present

Thematic Study across 750 Years · Medieval, Renaissance, 18th/19th Century & Modern Eras

Pupil Name: _____

Class / Set: _____

Target Grade: []

SECTION B PROGRESS & ASSESSMENT TRACKER (36+4 MARKS TOTAL)

Marked by Teacher or Peer Verified

Assessment Component & Stem Focus	Format Style	Max Marks	Pupil Score
Round 1 (Stepped): Q3 Similarity / Difference (Medieval vs Renaissance)	Stepped Ladder	/4	_____
Round 1 (Stepped): Q4 Explain Why (Surgical Progress c1700–c1900)	Stepped Ladder	/12	_____
Round 1 (Stepped): Q5/Q6 Statement Essay (Germ Theory vs Public Health)	Stepped Ladder	/16+4	_____
Round 2 (Dual-Track): Q3 Similarity / Difference (Great Plague vs Black Death)	Dual Track	/4	_____
Round 2 (Dual-Track): Q4 Explain Why (Penicillin Mass Production c1938–45)	Dual Track	/12	_____
Round 2 (Dual-Track): Q5/Q6 Statement Essay (Four Humours c1250–c1700)	Dual Track	/16+4	_____
Round 3 (Simulation): Section B Full Exam Hall Simulation (Q3, Q4, Q5/6)	Exam Pitch (55m)	/36+4	_____
TOTAL COMBINED EXAM PRACTICE MARKS:		/104	_____ / 104

Teacher / Peer Marker: _____

Date: _____

Overall Grade: [9 8 7 6 5 4]

SECTION B EDEXCEL EXAM FAST FACTS & STEM BLUEPRINTS (36+4 MARKS · ~55 MINS) OPTION 11 PAPER 1 SECTION B

- **Q3 Similarity / Difference (4 Marks · ~6 mins):** *P-F-C Formula*. State core **Point of Comparison** in sentence 1 → support with **specific evidence from Period 1** → support with **specific evidence from Period 2** → explain significance.
- **Q4 Explain Why (12 Marks · ~18 mins):** *3 P-E-E Paragraphs*. **Point** clearly linking to the question → **Evidence** deploying precise facts/names/dates → **Explanation** tracing the direct causal link. (Must use both stimulus points + own knowledge).
- **Q5/Q6 Either/Or Statement Essay (16 Marks + 4 SPaG · ~30 mins):** *Balanced Evaluation*. Point 1 (Agree with statement) → Point 2 (Counter-factor/Disagree) → Point 3 (Alternative factor) → **Criteria-Led Conclusion** explaining *why* one factor was more significant.

Question 3: Similarity & Difference Across Eras (4 Marks)

P-F-C Formula · ~6 Mins

Explain one way in which ideas about the cause of illness in the Medieval period (c1250–c1500) were similar to ideas about the cause of illness in the Renaissance period (c1500–c1700).

 **Level 1 Launchpad:** Starter: "One way in which ideas about the cause of illness were similar was the continuing belief in..."
Facts to Include: Persistence of miasma (bad air) and the Four Humours; despite Harvey and Vesalius disproving Galen on anatomy, ordinary physicians in 1665 still blamed the Great Plague on miasma and humours, just as in 1348.

One way in which ideas about the cause of illness were similar across both eras was...

In the Medieval period, people believed that...

Similarly, in the Renaissance period, (provide precise evidence from the 1665 Great Plague)...

This shows a strong continuity in medical thinking because...

 **Level 2 (Grade 9) Comparison Rule:** You must explicitly refer to BOTH periods with equal factual depth. Do not spend all your time describing the Medieval period and leave the Renaissance to a single sentence!

Question 4: Causal Explanation (12 Marks)

3 P-E-E Paragraphs · ~18 Mins

Explain why there was rapid progress in surgical techniques in the period c1700–c1900.

You may use the following in your answer: • James Simpson and chloroform • Joseph Lister and carbolic acid (You must also use information of your own.)

🔥 **Level 1 Launchpad:** Paragraph 1: Anaesthetics (Simpson 1847) solved pain; Paragraph 2: Antiseptics (Lister 1867) solved infection; Paragraph 3 (Own Knowledge): Aseptic surgery (sterilization, rubber gloves) and ligatures solved blood loss.

PARAGRAPH 1: STIMULUS FACTOR 1 — THE CONQUEST OF PAIN (SIMPSON & CHLOROFORM)

One major reason for surgical progress was the overcoming of pain through anaesthetics...

In 1847, James Simpson discovered that chloroform...

This transformed surgery because doctors could perform deeper, more complex operations...

PARAGRAPH 2: STIMULUS FACTOR 2 — THE CONQUEST OF INFECTION (LISTER & CARBOLIC ACID)

A second critical factor was the reduction of post-operative infection...

Inspired by Pasteur's Germ Theory, Joseph Lister used carbolic acid in 1867 to...

Consequently, mortality rates in his surgical wards plummeted from...

PARAGRAPH 3: OWN KNOWLEDGE FACTOR — ASEPTIC SURGERY & LIGATURES

Furthermore, surgery advanced beyond antiseptics through the development of aseptic surgery...

For example, (detail steam sterilization, Halsted's rubber gloves, or surgical catgut)...

⚡ **Level 3 (Grade 9) Analytical Glue:** Explain how Simpson's discovery initially led to the 'Black Period' of surgery (higher infection from operating deeper) *until* Lister's antiseptics solved the sepsis crisis!

Question 5 / 6: Either/Or Statement Essay (16 Marks + 4 SPaG) Balanced Evaluation · ~30 Mins

"Louis Pasteur's Germ Theory was the most significant turning point in the prevention of disease between c1700 and the present."

How far do you agree with this statement? Explain your answer.

You may use the following in your answer: • The 1875 Public Health Act • Fleming, Florey and Chain's development of penicillin (You must also use information of your own.)

Side A: Agree (Pasteur & Koch)

Disproved spontaneous generation; proved specific microbes cause specific diseases; unlocked scientific vaccines (rabies, anthrax) and modern hygiene.

Side B: Government Action

1875 Public Health Act made sanitation compulsory; clean water, sewers, and rubbish disposal saved far more working-class lives than laboratory science alone.

Side C: Modern Breakthroughs

Penicillin (1941) provided the first cure for internal bacteria; modern lifestyle campaigns (anti-smoking) and DNA (1953) target non-infectious causes.

INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (PASTEUR'S GERM THEORY)

To a significant extent, I agree that Louis Pasteur's Germ Theory was a profound turning point...

Prior to 1861, doctors believed in spontaneous generation, which meant...

By proving in his swan-neck flask experiments that germs caused decay, Pasteur enabled Robert Koch to...

Consequently, this transformed prevention by allowing scientists to develop targeted vaccines for...

PARAGRAPH 2: COUNTER-FACTOR — GOVERNMENT ACTION & PUBLIC HEALTH

However, it can be strongly argued that the 1875 Public Health Act was a greater practical turning point...

While Pasteur understood the cause in a laboratory, ordinary people were dying from filthy urban conditions...

By abandoning laissez-faire and legally forcing local councils to provide clean water and sewers...

Consequently, this administrative intervention saved millions from water-borne epidemics like cholera...

PARAGRAPH 3: MODERN ERA & OTHER FACTORS (PENICILLIN / DNA / PREVENTION)

Furthermore, in the 20th century, the nature of disease prevention shifted towards...

For example, while Fleming, Florey, and Chain's penicillin provided an unprecedented cure...

Modern prevention has increasingly focused on lifestyle campaigns and genetic understanding (DNA)...

CONCLUSION: CRITERIA-LED JUDGEMENT (WHY ONE FACTOR WAS DECISIVE)

In conclusion, while Pasteur's Germ Theory provided the indispensable scientific foundation for understanding disease...

The true turning point in national disease prevention was government public health intervention because...

⚡ SPaG Checklist (+4 Marks): ☐ Accurate spelling of key terms (Pasteur, bacterium/bacteria, antiseptic, laissez-faire); ☐ Sustained analytical tone throughout.

Question 3: Similarity & Difference Practice (4 Marks)

Cross-Period Comparative Matrix

Explain one way in which the government's response to the Great Plague (1665) was different to the response to the Black Death (1348).

Comparative Fact Vault (AO1)

1348 Black Death: Government action was minimal; King Edward III ordered street cleaning; local authorities were powerless; relied on church prayers.

1665 Great Plague: Mayor of London enforced strict quarantine; searchers identified infected; red crosses painted on doors; watchmen guarded houses; mass graves.

Difference Connectives

- "In contrast to 1348, authorities in 1665..."
- "While medieval response was largely fatalistic, Renaissance response..."

Level 2 Standard: Focus strictly on GOVERNMENT/AUTHORITY action in both periods.

One way in which the government's response was different was the degree of organized quarantine...

In 1348, during the Black Death, authorities took very little organized action because...

In contrast, during the Great Plague of 1665, the Mayor of London enforced strict regulations such as...

This shows that by 1665, authorities took a far more active, systematic approach to containment...

Question 4: Causal Explanation Practice (12 Marks)

Penicillin Breakthrough

Explain why there was rapid progress in the development and mass production of penicillin in the years c1938–c1945.

Stimulus: • Howard Florey and Ernst Chain • US Government funding and the Second World War

Causal Factor Vault (AO1)

Florey & Chain (1938–41): Purified Fleming's 1928 mould; tested on 8 mice (1940); human trial on Albert Alexander (1941) proved it killed infection in blood.

US War Funding (1941–44): British factories made bombs; US government funded deep fermentation tanks; enough penicillin to treat all Allied D-Day wounded (June 1944).

Own Knowledge: Cantaloupe melon mould strain found in Peoria, Illinois; corn steep liquor boosted yield 20x.

Level 3 Standard: Explicitly show how the pressure of WAR overcame the financial and industrial barriers to mass production.

PARA 1: FLOREY & CHAIN'S SCIENTIFIC BREAKTHROUGH

The primary scientific catalyst for progress was the biochemical work of Florey and Chain...

This was critical because while Fleming discovered penicillin accidentally in 1928...

PARA 2: SECOND WORLD WAR & US STATE INTERVENTION

However, scientific discovery could not achieve mass production without wartime urgency...

Recognizing its battlefield value, the US government poured millions of dollars into...

PARA 3: INDUSTRIAL TECHNOLOGY & AGRICULTURAL BREAKTHROUGHS

Furthermore, mass production was unlocked by industrial innovations such as...

Question 5 / 6: Statement Essay Practice (16+4 Marks)

Four Humours c1250–c1700

"The Theory of the Four Humours was the most significant foundation of medical knowledge between c1250 and c1700."

How far do you agree? Explain your answer.

Stimulus: • The enduring influence of Galen • Traditional herbal remedies

Continuity vs Change Matrix

Four Humours Dominance: Endorsed by Church; Galen's texts taught in universities; phlebotomy, purging, regimen sanitatis; used to explain 1348 and 1665 plagues.

Renaissance Challenges: Vesalius (1543) proved 300+ errors in Galen; Harvey (1628) proved circulation; Sydenham (1676) grouped diseases by symptoms not individual humours.

Level 4 Evaluation: Distinguish between theoretical breakthroughs in universities and the continuity of humeral treatments among ordinary people.

PARA 1: THE ENDURING DOMINANCE OF THE FOUR HUMOURS

On the one hand, the Four Humours was unquestionably the dominant foundation of medicine...

Supported by the Catholic Church, Galen's humeral theories were taught as indisputable dogma...

PARA 2: RENAISSANCE CHALLENGES TO HUMERAL THEORY

However, between c1500 and c1700, scientific empirical observation began to dismantle humeral ideas...

Vesalius proved Galen wrong on anatomy in 1543, and William Harvey proved...

 Alternative Factors Toolkit

Herbal Remedies: Practical folklore medicine passed down by women/wise-women; honey, mint, willow bark; independent of Greek theory.

Sydenham & Observation: Observed epidemics as external clinical entities rather than internal balance of humours.

 **Grade 9 Judgement:** Conclude that while anatomy changed rapidly, actual treatment remained fundamentally humeral until the 18th century!

PARA 3: TRADITIONAL HERBAL & ALTERNATIVE APPROACHES

Furthermore, for the vast majority of ordinary people, medical knowledge was founded on...

Herbal remedies and community care provided by apothecaries and local women...

CONCLUSION: SUPPORTED FINAL JUDGEMENT

In conclusion, I agree to a large extent that the Four Humours was the most significant foundation...

Although anatomical discoveries by Vesalius and Harvey laid the groundwork for modern science...

In terms of everyday clinical treatment, humeral bleeding and purging remained unchallenged until c1700...

SECTION B: MEDICINE IN BRITAIN, C1250–PRESENT

Round 3: The Planning Engine Room · Thematic Causal & Synoptic Deconstruction

ROUND 3: ENGINE ROOM

1. Cross-Period Comparison Engine (Q3 Practice):

Comparison Question	Period 1 Specific Evidence	Period 2 Specific Evidence
Treating Epidemics: Black Death vs Great Plague	1348: Flagellation, church prayers, carrying herbs, vinegar.	1665: Watchmen locked doors, searchers, burning barrels of pitch, plague pits.
Hospital Care: Medieval vs 19th Century	c1300: Monasteries (St Bartholomew's); focus on spiritual rest; no doctors.	c1860: Nightingale pavilion wards; trained nurses; antiseptic surgery.

2. Overarching Thematic Factors Matrix (Q4 & Q5/Q6):

Core Factor	Key Historical Examples Across 750 Years	Direct Analytical Impact
Government Action	1875 Public Health Act, compulsory smallpox vaccination (1853), NHS (1948)	Enforces sanitation and equal access beyond market forces.
Science & Tech	Printing press (1476), Microscope, Swan-neck flasks, X-rays, DNA (1953)	Provides empirical proof that overturns dogma.
Key Individuals	Vesalius, Harvey, Jenner, Simpson, Lister, Pasteur, Koch, Snow, Fleming	Acts as intellectual pioneers driving revolutionary change.

 **PRE-FLIGHT AUDIT CHECKLIST:** ☐ Did I compare BOTH eras with factual detail in Q3? ☐ Did I explain THREE distinct causes for Q4? ☐ Is my Q5/Q6 essay structured around criteria-led evaluation?

SECTION B: MEDICINE IN BRITAIN, C1250–PRESENT

Round 3: The Exam Pitch · Authentic Edexcel Section B Simulation (36+4 Marks · 55 Mins)

TIMED EXAM PITCH

AO1 Knowledge: Precise names, dates, individuals, and acts across all four eras. AO2 Analysis: Sustained explanation of similarity, causation, and relative importance.

Question 3 (4 Marks): One way in which hospital care in the 19th century differed from medieval hospitals was...

Question 4 (12 Marks): Explain why penicillin was mass-produced rapidly c1938–45:

Question 5 / 6 Essay (16+4 Marks): " " "

TOP 3 FATAL EXAMINER TRAPS TO AVOID FOR SECTION B

• The One-Period Comparison Trap

In Question 3, you cannot get above Level 1 if you only talk about one era. You must give balanced, explicit factual evidence for BOTH periods!

• The Overnight Miracle Trap

Never describe scientific breakthroughs (Vesalius, Harvey, Jenner, Pasteur) as instant cures. In reality, ordinary treatments took decades or centuries to catch up!

• The Storytelling Narrative Trap

Question 4 and 5/6 are causal and evaluative essays, NOT stories. Never just list what Jenner or Snow did; explicitly explain *why* it changed outcomes.

SPECIFICATION PRACTICE BANK: 100% THEMATIC CURRICULUM COVERAGE

Every remaining thematic specification bullet point tested below

1. Q3 Similarity (4m): Explain one way in which the training of physicians in the Medieval period was similar to the training of physicians in the Renaissance.

2. Q4 Explain Why (12m): Explain why government attitudes to public health changed in the period c1848–c1875. (Edwin Chadwick, John Snow).

3. Q5/Q6 Essay (16+4m): 'The founding of the National Health Service in 1948 was the most significant breakthrough in medical care between c1900 and the present.' How far do you agree?

4. Q3 Difference (4m): Explain one way in which treatments for the Black Death (1348) were different to treatments for Cholera (1854).

5. Q4 Explain Why (12m): Explain why understanding the causes of disease advanced rapidly after 1900. (Watson & Crick/DNA, lifestyle research into lung cancer).

⚡ **750-YEAR SYNOPTIC TITANS:** Hippocrates & Galen (Humours) → Vesalius & Harvey (Anatomy) → Sydenham (Observation) → Jenner (Vaccines) → Simpson (Chloroform) → Lister (Antiseptics) → Pasteur & Koch (Germs) → Snow & Chadwick (Public Health) → Fleming, Florey & Chain (Antibiotics) → Watson, Crick & Franklin (DNA).